

Ethics and Humanitarianism in Global Mental Health

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“A man without ethics is a wild beast loosed upon this world.”

Camus

“Act in such a way that you treat humanity, whether in your own person or in the person of any other, never merely as a means to an end, but always at the same time as an end.”

Immanuel Kant

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Abstract

This chapter addresses the intellectual history and definitions of ethics, humanitarianism, and global mental health. In recent decades there has been an increasing use of the terms ethics and humanitarianism; factors such as the events of World War II, the volume of immigration, political conflicts, and famine are often cited as driving the use of the terms “ethics” and “humanitarianism” (see Figs. 1 and 2 which depict the frequency of the use of the terms from a corpus of books in English) (https://books.google.com/ngrams/graph?content=Humanitarianism&year_start=1800&year_end=2019&corpus=26&smoothing=3&direct_url=t1%3B%2CHumanitarianism%3B%2Cc0). The historical, economic, political, and sociocultural background and aspects of globalization are mentioned, and the ethical implications of these factors are also discussed. A definition of global mental health is suggested. Five principal criteria are advanced for an activity to be considered as pertaining to global mental health. Potential negative and unethical practices even if considered humanitarian are cited. The chapter then concludes with examples of ethical missteps in such areas as education, research, training, and funding. Suggestions for minimizing negative ethical practices are made.

Introduction and Background

In recent years there has been greater attention on the ethical and humanitarian issues around global mental health. This is to be expected as the array of activities considered to be pertinent to global

health and mental health is increasing and hence more visible. Other factors include the elements of power asymmetry and of limited autonomy that may occur between donor nations and entities and the recipient ones and between funding agencies and the recipient agencies. Who calls the shots and the questions of what, for whom, and by whom become relevant? Also, crucial questions of the setting of proper intervention services, funding, and research are explored. This increased awareness accompanying global health and mental health activities has to some extent been justified by human rights issues; issues of access to care; the relationships between providers and users, donors, and recipient entities as mentioned; and the greater need for efficiency, accountability, and cultural sensitivity. So is the changing definition of disability and increased attention on improving the quality of life of the clinically “disabled” individual? Similarly, the definition of health has been expanded to include issues related to food and water security as well as climate change.

Definitions

Ethics is defined in Merriam Webster as “rules of behavior based on ideas about what is good or bad” (Merriam Webster [n.d.](#)). There are many schools and theories of ethics, and each school has a special focus, e.g., self-interest, compassion, pleasure, and obligations. Sometimes the ethical theories or frameworks are complementary, and at other times, they may be opposite of another theory. Peter Singer, an Australian-born prominent ethicist, observes “ethics is not a matter of factual knowledge in a way the sciences and other branches are, rather it has to do with determining

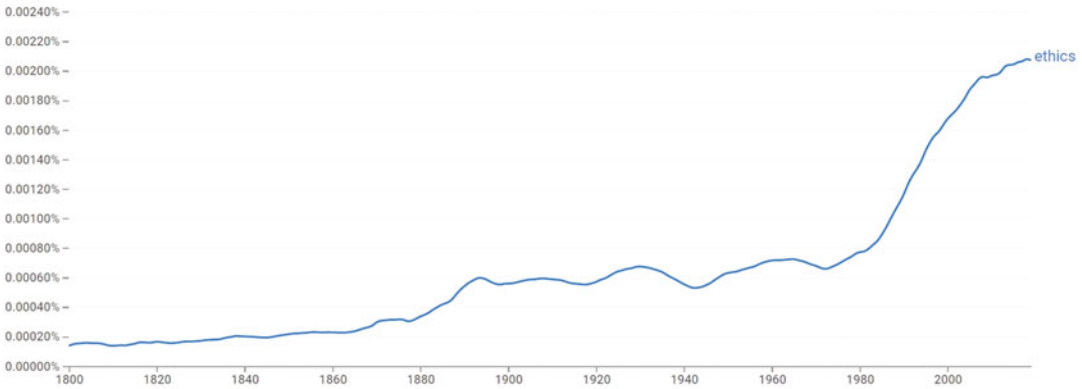


Fig. 1 Google Books Ngram Viewer ethics

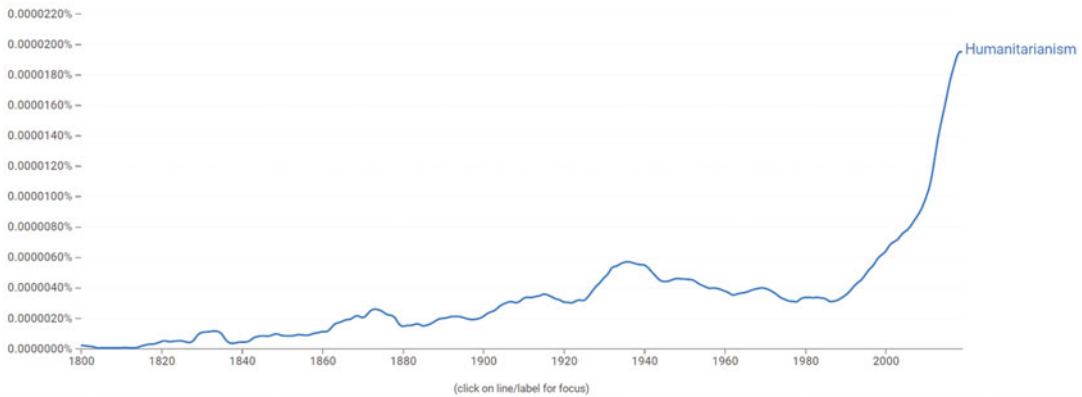


Fig. 2 Google Books Ngram Viewer humanitarianism

the nature of normalizing theories and applying these sets of principles to practical moral problems (Singer 1985).” Ethics are derived from the values, beliefs, and principles of various cultures, traditions, and religions. Ethics are not static and may vary from one historic period to another and from one community to another. Recalling, for example, the impact in the 1970s of the Watergate scandal during the Nixon era where Nixon was found to have been implicated in the break-in of democratic policy headquarters in Washington, D.C., many lawyers were involved in this, and it resulted in the focus among the legal community on the need for proper training of law students in ethics. Other examples were the Enron and Parmalat business scandals. This similarly alerted business schools to pay closer attention to the training of their wards from an ethical point of

view. An example of cultural variation has to do with autonomy and individual choices. An example will be end of life decisions. In many traditional societies and ethnic groups, the decisions surrounding end of life and the use of power of attorney rest with the family even in the USA. Among some other cultures including whites in America, the decision rests with the individual rather than the family. The American culture and the western culture appear to emphasize individual decision versus a family-oriented decision.

Some philosophers use the terms ethics and morality interchangeably. Others see a difference and would emphasize ethics as the branch of philosophy that deals with an exploration through reasoning as to what is right or wrong and good or bad, and morals are seen as the practice of rightness or wrongness of an individual’s or a group’s

actions. Ethics or ethical theories may be classified depending on their central focus or assumptions. With respect to this chapter, the most relevant theories are the normative, meta-ethics, applied, and descriptive ethics. In philosophy, normative ethics are broken down into three types: deontological, consequential, and virtue ethics. Deontological ethics is based on rules or obligations as may be found in professional codes of ethics, for example. Consequential ethics is based on outcomes, e.g., should a military officer order the bombing of a village of 100 people in order to save 10 prisoners of war? Virtue ethics are derived from the teachings of Aristotle, who encouraged the practice of certain behaviors and habits with a view to becoming a moral and happy person. Further, applied ethics refers to the application of normative ethics to practical situations and dilemmas, and meta-ethics refers to the study of some fundamental assumptions on which concepts of ethics are based to the field of ethics or moral philosophy. What do we mean by good vs bad, what is rightness? What are meanings of these terms? Descriptive ethics refers to an exploration of ethical practices of a group or community.

Related to the above theories of ethics are frameworks of ethical principles. There is always the nagging question as to whether there are any universal principles of ethics. An instinctive answer will be to say yes with respect to the value that thou shalt not kill. However in human history wars abound, violence is a commonplace. Nevertheless, there have been attempts to distill ethical principles into sometimes four basic principles. Core framework is that they suggest four basic principles as being autonomy, beneficence, non-maleficence, and the principle of justice. This framework has sometimes been expanded to 12. This framework has sometimes been expanded to include for example, 7, 8, and even up to 12 principles by various professional organizations, guilds, and as called for by the ethical situation. For example, while bioethicists accept the four basic principles for case managers, additional principles include veracity (truthfulness) and fidelity (trust). For nurses the framework is expanded to include additional principles, i.e., accountability. For centuries physicians were

guided by the Hippocratic Oath that can be summarized as follows: (I) solidarity with teachers and other physicians, (II) beneficence (to do good or avoid evil) and non-maleficence, (III) not assist suicide or abortion, (IV) leave surgery to surgeons, (V) do no harm, (VI) not seduce, and (VII) maintain confidentiality and never gossip. With societal, technological, and legal changes, this Hippocratic Oath has seen major modifications. For Jewish populations, the code of ethics is influenced by Maimonides, prayer, and wisdom. In short, therefore, ethical principles change across time and place. This fact coupled with the multidisciplinary and multi-professional nature of global health and mental health instills the need to be ethically aware, sensitive, and ethically correct.

“Humanitarianism,” like “ethics,” are terms that have experienced an upsurge in usage as depicted in Fig. 2 (https://books.google.com/ngrams/graph?content=ethics&year_start=1800&year_end=2019&corpus=26&smoothing=3&direct_url=t1%3B%2Cethics%3B%2Cc0#t1%3B%2Cethics%3B%2Cc0). Merriam Webster defines humanitarianism as “the promotion of human welfare” (Red Cross and Red Crescent [n.d.](#)). It is suggested that the exponential increase in its use is attributable to events since the 1930s: World Wars, the Holocaust, the Biafra War, the floods of Bangladesh, the Indonesian Tsunami, the Ethiopian famine, and of course the continued political crisis, the volume of immigration and refugee problems. The Red Cross and the Red Crescent have been beacons for humanitarian crisis relief and intervention. Their founding is attributed to Henry Dunant, a Swiss business man and humanitarian. He had witnessed firsthand a very bloody battle at Solferino in Italy, where the French and Italians were attempting to drive the Austrians out of Italy. The Red Cross was founded in 1858. Dunant advocated initially for the training of volunteers to offer assistance to war-wounded soldiers and subsequently to provide national relief. The Swiss-based committee became the International Committee of the Red Cross with the designated flag. The corresponding flag for Muslim countries was the Red Crescent. In 1919 the International Federation of Red Cross and Red Crescent Societies was founded (<https://www.icrc.org/en/doc/assets/files/publications/icrc-002-1067.pdf>). The vision and mission have been

expanded from providing care and assistance to wounded soldiers to providing relief for disasters, assisting immigrants and refugees, and other similar activities. The activities of the International Red Cross and Red Crescent Movement are guided by a code of conduct. NGOs or other relief agencies that work in disaster relief are also guided by this code. The essential principles of the code are as follows:

- 1) The humanitarian imperative comes first.
- 2) Aid is given regardless of the race, creed, or nationality of the recipients and without adverse distinction of any kind. Aid priorities are calculated on the basis of need alone.
- 3) Aid will not be used to further a particular political or religious standpoint.
- 4) We shall endeavor not to act as instruments of government foreign policy.
- 5) We shall respect culture and custom.
- 6) We shall attempt to build disaster response on local capacities.
- 7) Ways shall be found to involve program beneficiaries in the management of relief aid.
- 8) Relief aid must strive to reduce future vulnerabilities to disaster as well as meeting basic needs.
- 9) We hold ourselves accountable to both those we seek to assist and those from whom we accept resources.
- 10) In our information, publicity, and advertising activities, we shall recognize disaster victims as dignified humans, not hopeless objects (The Resilience Paradigm [n.d.](#)).

The code of conduct emphasizes humanity, impartiality, neutrality, and independence. Suffering is to be addressed wherever it is found. In terms of partiality, humanitarian action is to be based on need alone and not on factors such as nationality, race, religious belief, or political opinions. Humanitarian actors in hostility must be neutral in discharging their jobs and not engage in political, religious, or other controversies. The actors must exercise independence from other objectives, e.g., political, military, or economic. This code of ethical conduct aims to engender safety of the actors and trust in them. Historically the dominant paradigm in humanitarianism has

been the Dunantist or classical paradigm which is central to the work of the UN humanitarian agencies and NGOs. More recently, that approach has been challenged by a newer paradigm, the resilience paradigm (The four dimensions of globalisation [n.d.](#)). This differs from the old paradigm in a variety of ways. One difference is that in the resilience paradigm, there is an attempt to unite relief to rehabilitation to development. Relief assistance was redesigned to build on local communities' strengths and capabilities, and the local people were seen as the first responders. It has been suggested that in classical humanitarianism, the focus tends to be short-term crisis, and local institutions are treated with mistrust and ignored sometimes with the excuse that they require capacity building. Both paradigms have their strengths and weaknesses, but it is perhaps fair to say that the new paradigm – resilience humanitarianism – is more relevant in stimulating local autonomy and in terms of disasters such as those due to long-term crisis, e.g., climate changes.

Global health and global mental health are two movements that are derived from the process of globalization. There are a multitude of definitions for globalization. My preference is the definition by Giddens. It is appealingly succinct:

“the intensification of worldwide social relations which link distant localities in such a way that local happenings are shaped by events occurring many miles away and vice versa.” (Koplan et al. [2009](#))

Koplan defines global health as:

“an area for study, research, and practice that places a priority on improving health and achieving equity in health for all people worldwide. Global health emphasizes transnational health issues, determinants, and solutions; involves many disciplines within and beyond the health sciences and promotes interdisciplinary collaboration; and is a synthesis of population-based prevention with individual-level clinical care.” (Patel and Prince [2010](#))

The driving factors for globalization include increased worldwide connectivity, easier and more rapid communications, and a greater sense of recognition that we are all together, e.g., with the effects of climate change. Although too numerous to count, all definitions of

globalization comprise an overlap of themes: facilitation of trade and commerce worldwide, humanitarianism, and access to a playing field. Perhaps by serendipity or otherwise, international soccer has often been seen as an example of globalization. This refers to breakdown of barriers in the recruitment and movement of talented foreign players, the relaxation around migration laws, and the universal acceptance and appreciation of soccer rules. Like the wider movement of globalization, international soccer has experienced some untoward consequences. This is exemplified by the concentration of talented players in the wealthier leagues and the corresponding dominance of the leagues in international and national soccer.

Koplan emphasizes a number of facets including transnational health issues, various health determinants, and possible solutions. His definition also implies a multidisciplinary approach to this area of study, research, and practice as well as a synthesis of population-based prevention and individual-level clinical care. This definition alerts us to the ethical and humanitarian implications for global health.

Patel and Prince define global mental health by borrowing from the definition by Koplan and substituting “mental health” for “health” (Okpaku 2014). I have since defined global mental health by emphasizing some principal criteria. These are universal and transnational criterion, public health criterion, stakeholders’ criterion, problem ownership criterion, and team criterion (<https://www.imf.org/en/News/Articles/2015/09/28/04/53/sp052197>). For a project to be deemed a global mental health project, it should meet the above definitional criteria.

Ethics, Humanitarianism, Global Health and Global Mental Health, and the Sustainable Development Fund

The above definition of global mental health alerts us to the role of ethics and humanitarianism in global health and global mental health. Clearly, a driving factor of foreign and global

health and mental health is humanitarianism. However, that is not all. Governments, NGOs, and other entities which engage in foreign aid and collaboration as in global health and global mental health may be motivated by self-interest, economic motives, and power dynamics; thus their actions must also be subject to ethical scrutiny and consideration. Similarly, of course, ethics is key, in the humanitarianism arena during a variety of crisis. Examples of these crises are military actions and wars, famine from climate change, or disruptions from natural disasters. It is important to examine the motives of both donor and recipient organizations under such circumstances. With respect to universal/transnational criteria, the problem or issue should be of transnational relevance. Examples of such issues are the mass stigmatization of mentally ill individuals or worldwide poverty. For the public health criterion to be met, the problem should have a population basis. An example might be violence as a public health issue. In terms of the problem ownership criterion, the problem should be owned by the recipient organization, institution, or country. The team criterion implies that the teams engaged in the project are multidisciplinary and multiparty. This definitional approach affords us a platform to examine to what extent a project or arrangement is ethical or not. If the definitional criteria are not adherent to, then there is a red flag. Another level of analysis is afforded us by an examination from a dimensional point of view. These dimensions are historical, economic, political, and sociocultural dimensions.

Historical Dimension

In a substantial number of global health arrangements, the recipient countries have had a colonial past history with the donor country. Examples of such relationships are Nigeria, Ghana, Kenya, and the UK. The consequences for such relationships for the developing countries are the potential for increased suspicion of exploitation and of increased sensitivity toward the actions of the superpowers. “I fear donors when they bring

gifts” (Trojan War). In this regard, some writers have labeled globalization as “new colonization” (Alindale et al. 2002). In other words, globalization can be seen as continued foreign domination and exploitation.

Economic Dimension

In theory, one mission of the International Monetary Fund and the World Bank is the eradication of poverty worldwide as well as better quality of life worldwide. However, the results in practice are not always so. In fact, there is evidence to suggest that IMF interventions and some consequences of globalization have actually led to increased poverty in Africa where foreign direct investments have fallen (UNC7AD2012) (Appadurai 1996). There have been other examples as well. For example, in the Asian crisis of 1997, many Asian countries such as Indonesia and Malaysia suffered a significant recession on what was initially a minor problem. Argentina suffered a similar fate in 2001.

Political Dimension

While foreign aid can be the result of humanitarian intention, this may not preclude a donor country from acting in its political self-interest and dominance, for its economic advantage, or power dynamics. Important aspects of the political domain are health, development, and security. Corresponding issues include the prevention of pandemics and access to affordable pharmaceuticals, e.g., the availability and distribution of vaccines. This is evident under the current crisis of COVID-19 where many developing countries don’t have adequate access to vaccines, and even within the European Union, there are problems with distribution and availability. In other words the primary aim for foreign aid may be political or economic rather than for humanitarian reasons. Brazil’s declaration of the right to health care as a human right is worthy of mention. Inevitably, poor health is directly linked to loss of economic and political viability.

Sociocultural Dimension

Similarly, to the definitions by Giddens, Appadurai (1996) saw globalization as “a process of cultural mixing and hybridization across locations and identities” (Millennium Development Goals (MDGS) 2007). The internet, social media, and mass communication have made it possible to negotiate across countries and cultures in real time. A good example is the rescue operation of the Chilean mine disaster (October 13, 2010) which was vividly portrayed in news media.

This discussion would be incomplete without making reference to two major instruments of the UN. These instruments are the Millennium Development Fund of 2000 and its successor the Sustainable Development Fund, both of which are fully embedded in ethical and humanitarian principles. In 2000, 80 heads of state signed the Millennium Declaration and committed their governments “to a collective responsibility to uphold the principles of human dignity, equality, and equity at the global level” (UN Sustainable Development Goals n.d.). The document specified eight goals to be achieved by 2015. These were as follows: (a) end poverty and hunger, (b) promote access to universal education, (c) promote gender equality, (d) improve child health, (e) improve mental health, (f) combat HIV/AIDs, (g) promote environmental sustainability, and (h) promote global health. Country performance of these goals by the end of 2015 varied. The performance of low-income countries lags behind those of high- and middle-income countries in some indices. For example, 74% and 59% of low- and middle-income countries are on track for reducing child mortality and maternal mortality, respectively, compared with 69% and 22% using global MDGs. Only 20% and 7% of low- and middle-income countries are on track for the child and maternal mortality fast-track targets. The Millennium Development Fund was followed by the current Sustainable Development Fund, containing 17 goals which include good health and well-being and no poverty. Additional goals are clean water and sanitation, gender equality, reduced inequalities, responsible

consumption and production, climate action, and peace justice and strong institutions (NEPAD 2001). Clearly these two instruments idealize a confluence of human goals and activities of well-being, equality, and justice.

Although the future of global health and mental health is difficult to predict, especially under the current COVID-19 pandemic, the mechanism by which these processes are carried out remains one of globalization, a dominating process. However, the results have not completely met the idealized objectives, and indeed some countries have been further marginalized, and results are not universally accepted. Witness, for example, the frequent protests and demonstrations that occur during the meetings of the International Monetary Fund. Also, consider the following statement by an influential arm of the African Union – the New Partnership for Africa’s Development (NEPAD). The group NEPAD stated:

“In the absence of fair and just global rules, globalisation has increased the ability of the strong to advance their interests to the detriment of the weak, especially in the areas of trade, finance, and technology. It has limited space for developing countries to control their own development, as the system makes no provision for compensating the weak. The conditions of those marginalized in this process have worsened in real terms. A fissure between inclusion and exclusion has emerged within and among nations.” (Wolfensohn 2001)

An ex-President of the World Bank Wolfensohn also had his reservations in the following comment, “We cannot turn back globalisation. Our challenge is to make globalisation an instrument of opportunity inclusion—not of fear and insecurity. Globalisation must work for all” (Swedish Ministry of Foreign Affairs 2003). Meanwhile a number of developed countries have inserted health into their foreign policies. By so doing, they demonstrate and achieve some ethical goals and humanitarian objectives. We will now examine the principal tenets of some foreign policies with health as an aspect of that policy. The activities of the Scandinavian countries have been prominent in this regard. They appear to focus on issues related to human rights, equity, and development. For example, in 2003 the

Swedish Parliament passed the bill *Shared Responsibility*. This bill was driven from a human rights perspective, and it focuses on the reduction of poverty in poor countries. It was a policy for development that was multisectoral, and it emphasized equity and sustainability for global development (Norwegian Ministry of Foreign Affairs 2012). In 2012, the Norwegian Ministry of Foreign Affairs published a policy document similar to the Swedish. The major goals of the policy document were to fight poverty, support and promote health sciences, reduce social inequalities worldwide, promote women’s rights and gender equality, and help reduce disease burden (HM Government 2008).

Health is Global was a policy document published by the UK government in 2008 (Centers for Disease Control and Prevention 2012). The stated objectives of this British Foreign Policy were better global health security; stronger, fairer, and safer systems to deliver health; more effective international health organizations; and stronger, freer, and fairer trade for better health to strengthening the way we use evidence to improve policy and practice.

US foreign policy emphasized health as a critical aspect of national security. The policy aims to collaborate with foreign governments and partners to achieve peace and security. The USA has contributed considerable amounts of money to the fight against HIV/AIDs and tuberculosis through the *Global Leadership against HIV/AIDs Fund and the Tuberculosis Reauthorization Act of 2008* (US Congress). The US Centers for Disease Control and Prevention (CDC) plays a major role in executing the US policies that strive to achieve the following objectives:

- Goal 1: Health Impact – improve the health and well-being of people around the world
- Goal 2: Health Security – improve capability to prepare for and respond to infectious diseases, other emerging health threats, and public health emergencies
- Goal 3: Health Capacity – build country public health capacity
- Goal 4: Organizational capacity – maximize the potential of CDCs global programs to achieve

impact (Federal Department of Foreign Affairs, Federal Department of Home Affairs 2006)

For the Swiss government, its policy objectives emphasize stakeholder participation, the rule of law, and human rights. In the Swiss policy, the term “developed” is broadened to include living in dignity (BRICS Health Ministry 2011). In addition to individual nations’ foreign policy that includes an international global health as an objective, there are other foreign aid arrangements. These arrangements could be regional, multinational, or bilateral. An important multinational arrangement is the BRICS states. This group includes Brazil, Russia, India, China, and South Africa. The particular strength of this group is that member countries have an expanding economy and segments of their populations live in conditions that are frequently found in developing countries. Their policy emphasis is “health for all.” Another relevant issue is the agreement for advanced countries to donate at least 0.7% of their GNP to foreign aid (OCED 2005). Often only five or six of these countries meet that target.

To recap, major themes of the above policies include issues of equality, fairness, equity, the protection of human rights, and the eradication of poverty. These issues encompass matters of ethics and humanitarianism as they relate to “right” and “wrong,” “good” and “bad,” (ethical issues) and improving the social welfare of communities (humanitarianism). So, what are the ethical and humanitarian challenges to global mental health? From what we have so far discussed, we can easily see that this is a minefield. However, we can be more systematic by viewing the issues from a definitional and dimensional point of view whereby we can easily discern the complexity of ethical and humanitarian factors in global mental health. Global mental health thus becomes a minefield of ethical dilemmas. Issues of autonomy, transparency, and accountability become prominent as we talk about stakeholders and problem ownership. Professional codes of ethics are relevant in team approaches. Similarly, there are other concerns from the dimensional point of view. Previous colonial powers may not exert

undue advantage over their previous colonies in matters of foreign aid. Paternalistic attitudes toward former colonies have to be discouraged. From the political point of view, donors and their agents should be sensitive to the boundaries of their political, economic, and self-interest as distinct from purely humanitarian goals. The socio-cultural dimension to ethics is derived at least in part from traditions, values, and culture. Global mental health activities are carried out within a sociocultural context be it a humanitarian crisis, a conflict situation, a research agenda, or a training and psychosocial intervention. The ethical priorities in each situation may vary. All of these are made more complex by issues of local values, resources, and cultural criteria. These arrangements may be bilateral, multisectoral, governmental, NGOs, or various permutations of these agencies. These issues are compounded by the salience of the context in which the global mental health activity and intervention are being carried out. The role of culture cannot be over-emphasized. Take but a few examples: (a) an American nurse who goes to a West African country and takes pictures of hospitalized mental patients, (b) the 23-year-old recently graduated American who goes to a Southern African country and attempts to give family planning advice to a 65-year-old chief, or (c) the young medical student who says he carried out cardiac surgery in an African hospital. A more positive example is the appropriateness of using smart diplomacy versus muscle diplomacy in negotiating foreign aid. Clarification of the above points is illustrated by the following examples. In analyzing each of them, the reader is encouraged to reflect on which ethical principle is being violated.

Priority Setting Planning and Monitoring Foreign Aid

Priority setting, planning, execution, and monitoring of foreign aid are potentially replete with ethical violations and missteps. To be done properly, they require the careful consideration of the asymmetrical nature of the power dynamics involved as well as the corresponding benefits.

Some aspects of this have been alluded to in my definition and criteria for global mental health which stresses the need for recipient entities – country, organization, or institution to own the problem. In addressing this issue, the Organization for Economic Cooperation and Development (OECD) formulated a declaration designed to promote accountability by both developed and developing countries in the delivery, monitoring, and management of foreign aid. The declaration is the Paris Declaration (OECD 2005) which has five principal goals (Sopko 2013). These are as follows:

- Ownership – partner countries exercise effective leadership over their development policies, strategies, and coordinate development actions.
- Alignment – donors base their overall support on partner countries' national development strategies, institutions, and procedures.
- Harmonization – donor's actions are more harmonized, transparent, and collectively effective.
- Managing for results – managing resources and improving decision-making for results.
- Mutual accountability – donors and partners are accountable for development results.

A useful template was suggested by Sopko in 2013. Sopko was a former US Special Inspector for Afghanistan Reconstruction. He suggested that to make foreign aid more relevant and useful, the following questions should be asked by the donor country:

- (1) Does the project or program make a clear and identified contribution to national interests or strategic objectives of donor countries?
- (2) Does the recipient country want or need the project?
- (3) Has the project been coordinated with other donor implementing agencies, the recipient government, and other international donors?
- (4) Are security conditions favorable to effective implementations overnight?
- (5) Are there adequate safeguards to detect, deter, and mitigate corruption?
- (6) Does the recipient nation have the financial resources, technical capacity, and political will to sustain the program?

- (7) Have the various stakeholders established meaningful and measurable metrics for determining success (<https://www.int/bulletin/volume/87/2/08.062695/en>)?

Also, the World Health Organization has published some guidelines which are useful in minimizing missteps and violations of ethical codes. The two most important would appear to be (a) ethics in epidemics, emergencies, and disasters (https://apps.who.int/iris/bitstream/handle/10665/44783/9789241502948_eng.pdf?sequence=1) and (b) a training manual: Ethical Standards and Procedures for Research (Khan and Najam 2009a). Again, we have to remember that the World Health Organization as a union agency is restricted to little more than consultation and monitoring. The above guidelines can be augmented by individual state regulatory bodies as well as the ethical guidelines in professional organizations. Under this heading, the suggestions by Najam and Khan are relevant: (<https://www.who.int/news.item/05-10-2020>) we need to pay attention to issues of exclusion and inclusion of stakeholders and the growth of data collection and information. Finally, we should support the idea of a level playing field to reduce the asymmetry in arrangements such as those between advanced donor countries and developing recipient countries, donors and recipients, and health providers and patients.

Funding for Mental Health Services

Mental health services tend to be less resourced than other aspects of health, both in funding and in staffing. This discrepancy is more appalling when we consider that low-income countries carry about 50% of the global burden of disease but spend only 2% of global spending (<https://www.oecd.org/financingsustainable>). It is not unusual when advocating for mental health services that senior administrative officers prioritize physical health and other issues over mental health services. In terms of staffing in low-resource countries, there are countries with less than a total complement of ten psychiatrists for the whole country. Another important issue has to do with

the Official Development Assistance (ODA). This agreement commits many developed countries to contribute a target of 0.7% of Official Development Assistance/Gross National Income (ODA/GNI) and 0.15–0.2% of ODA/GNI to the least developed countries. In the last few years, only about five to seven countries exceeded this UN target for international aid spending. In 2017 the following countries were the only ones to exceed the target: Denmark (0.85%), the Netherlands (0.75%), Norway (1.05%), Luxembourg (0.95%), and Sweden (1.4%) (Medicinal Drugs in the Third World 1987).

Pharmaceutical Practices

Some pharmaceutical companies have been known to violate ethical standards in their practices and some have in fact been charged with criminal violations. Firstly, there is a disparity in that although 70% of the world's population lives in the developing world, it produces only 7% of pharmaceuticals that it uses (Wise 2001). In addition to this, there are clear unethical violations which range from dumping of expired drugs to overpricing, bribery, corruption of government officials, and the use of medications that will under normal circumstances not be used in more affluent countries. Another area of concern is in regard to violations in clinical trials. One such example is a case in which Pfizer was charged for illegal practices. This action was due to clinical trials during a serious cerebral meningitis epidemic in Kano State in Northern Nigerian. The charges were as follows:

- 1) Lack of appropriate informed consent.
- 2) Some subjects (children) were given less than recommended dosages of medications.
- 3) Inappropriate medical record keeping.

Many children in the experimental antibiotic group suffered from paralysis, slurred speech, and brain damage resulting in about 6% mortality in the study group. The case was settled by Pfizer with monetary payments to Kano State as well as to the parents of some of the affected children

(Jenkins et al. 2010). Another set of pharmaceutical ethical concerns are related to the HIV/AIDS epidemic. Millions of Africans died from this condition because of lack of accessibility to effective drugs or lack of treatment due to stigma (Annas and Grodin 1998). An example of a bioethical issue in this situation is related to studies of maternal–child transmission of HIV/AIDS. Although AZT was already known to be effective, international studies were carried out with a placebo group as well as a short-term versus a long-term protocol (Luna 2004). The major criticisms centered around (a) unavailability of an effective agent to sick pregnant women and (b) the use of a placebo group in the presence of a proven effective agent (World Medical Association n.d.). Another major area of ethical concern relating to pharmaceutical practices centers around the availability of psychotropic medications. One aspect of this has to do with the low priority assigned to mental illness. This results in low budgets for pharmaceutical products for the treatment of mental disorders. This lack of availability sometimes involves the use of older psychotropics. One result is that resupplying or continuation of medications is not guaranteed. Sometimes organizers of medical missions worry as to the supervision of these medications when the physicians depart from their homes. My response to that is that one of the most pernicious instances of an imprisonment of the mind is a psychotic break or severe depression. A temporary relief of these conditions with donated samples may result in some temporary improved functioning. Also, in many instances in the absence of qualified psychiatrists, many experienced nurses and family practitioners in developing countries do have the sufficient expertise to maintain psychotropics. Another area of concern is excessively bureaucratic frameworks for the approval of imported drugs to developing countries. As administrative guidelines may be too bureaucratic, this leads to time wasting and the diversion of drugs or their deterioration in warehouses. Sometimes a country's formulary may be too restrictive and out of date due to limited budgets. In recent years, authorities in developing countries have discouraged the unethical practices by pharmaceutical companies

of attempting to influence the prescribing habits of health providers by making monetary or non-monetary gifts. We do not know the extent of such behavior in developing countries where because of financial or economic pressures the temptation may be greatest.

The above concerns, especially the ones relating to clinical trials in HIV/AIDs and the lack of attention to the proper use of placebo controls, have probably led to stricter and modified guidelines than the Declaration of Helsinki which was adopted in Edinburgh in 2000 – it has since been modified by the Council for International Organizations of Medical Sciences (<https://theeyeoffaith.com/2013/11/05/manic-monday-charcots-medical-muses/>). The use of appropriate informed consent, the inclusion and engagement of local collaborators, as well appropriate consultation may help to reduce some of the above ethical missteps.

Advocacy, Humanitarianism, and Use of Photography

Advocacy and humanitarianism are an essential aspect of global mental health. A prediction is that with decreasing availability of funding and resources, religious organizations, including the conservative ones, may assume a greater role in global health and mental health. This may be in disasters and emergencies. Under such conditions, the humanitarian principle that opposes a quid pro quo for assistance with proselytizing is to be strictly observed. Promotion of awareness is essential in advocacy and humanitarianism. This is sometimes done through the use of posters and photographs. There is a debate about the use of photographs of mentally ill individuals especially African or Asian, to highlight the problem. A closely related invasive practice is the photography of mentally ill individuals in institutions or state hospitals. I have previously mentioned above the case of a young US nurse who went to a West African country. She took pictures of a young man in a psychiatric hospital. Upon her return, she shared her “trophy” pictures of a sick individual with her colleagues and friends. When

I asked her why she took photographs of this individual, she replied that the individual had expressed to her his wishes to visit the USA, and in their conversation, the idea of taking his photograph came. The manipulation of patients is not new. Charcot was criticized for manipulating his female patients to assume certain attitudes and postures to further his theory of hysteria (Cox et al. 2017). The photography of mentally ill individuals, even with their consent or for promotional purposes even for advocacy, is an ethical issue. We are reminded a command based on words of *Jesus* in the *Sermon on the Mount*: “All things whatsoever ye would that men should do to you, do ye even so to them.” Individuals who support the practice often refer to its usefulness in raising awareness of mental illness and fundraising. If photographs are needed for advocacy, fundraising, and other humanitarian purposes, there are photographic technologies to conceal the identity of the individual patients.

Research

With increased emphasis on the accountability and effectiveness of interventions, research in global mental health is a major activity. Also, for individuals and institutions interested in global mental health, the opportunity for research and collaboration is an attraction. An ethical concern therefore is the setting of priorities and topics to be researched. Ethics has many gray areas, especially as its relevance within different cultures and under different circumstances may entail different issues. Appropriate ethical considerations in routine interventions may be markedly different from those in emergency conditions, especially in low-resource environments. The previously cited example concerning the clinical trials of AZT and the prevention of maternal–child transmission of HIV/AIDs is one example. Another important ethical consideration in global mental health research is the use of appropriate informed consent. This should be in an appropriate local language, understandable to the subjects and their powers of attorney. Community and local leaders should be involved. The privacy of information

and the use of data collected should be matters of concern. Researchers should be fair in the sharing of research data and in the publications. Colleagues in developing countries frequently complain of overseas collaborators who do not invite their participation in the use of the results of the studies. They feel they have been exploited. If the protocol also includes sharing of results with study subjects and local and community leaders, this step should not be omitted.

Brain Drain

The brain drain, or the migration of trained personnel from less developed countries to more advanced countries, pose a number of ethical dilemmas related to individual autonomy and right of movement, commitment and responsibility to the home country, and the moral obligation of the recipient country. This is indeed a personal and somewhat intractable problem. Because of the volume of the brain drain, there have been several proposals and attempts to curb it. A study indicated that 27–30% of physicians in the major destination countries of the USA, the UK, Canada, and Australia are international graduates (Tankwanchi et al. 2011). It is believed that the phenomenon may be increasing even as attempts are made to mitigate it. Its driving forces include easy labor migration, work, economy, and income conditions in home countries, demographic changes, the right income, and some consequences of globalization. An easily forgotten fact is the legacy of colonial history upon developing countries, where training and the recognition of postgraduate certifications and availability are linked to colonial relationships. The absence of available postgraduate training in low- and middle-income countries and the rights of individuals to provide the best for themselves and their families versus home countries' needs, against a background of political activity and unrest, poor economic circumstances and lack of security for a satisfying professional experience, create tensions. A survey of data from the American Medical Association indicated that there were 17,376 physicians who were identified as

having an African background and about 2/3 were trained in medical schools in Nigeria and South Africa. Jenkins also observed that a number of psychiatrists registered to practice in the destination countries were from low- and middle-income countries (LMIC) (Ndeti et al. 2004). Some of them are willing to work in less desirable placements such as state mental hospitals or transfer to less desirable specialties. A sobering figure for touch is as follows: the rate of psychiatrists to population in the UK rose from 5.9/100,000 in 2003 to 7.6/100,000 in 2013. The corresponding figure from Africa was 0.1/100,000 in 2014. Ndeti has suggested that the cost of training and investment expended by the LMIC should be reimbursed by the destination country. He attempted to calculate the economic loss to the LMIC of a trainee who then migrates abroad (WHO n.d.-c).

There have been other strategies to deal with the issue of brain drain, with varying efficiency and results. For example, in the 1970s in the UK, there was a guideline for consultant physicians to encourage foreign trainees and staff to return to their home countries upon obtaining their postgraduate certifications. Some countries insist on medical graduates completing national service commitment. Other countries, like the USA, may approve only restricted visas for foreign graduates and apply conditions to the renewal of these visas. South Africa has taken steps to ban the recruitment of physicians from other sub-Saharan countries. The World Health Organization now has a set of guidelines in an attempt to ease this problem. The guidelines are outlined in the WHO Global Code of Practice on the International Recruitment of Health Personnel (Hurst and Thompson 1993). This is to encourage human capacity building based on enhanced and other clinical environments and strategic recruitment and retention approaches. The issue of brain drain is an example of an ethical dilemma that deals with the autonomy of individual physicians and the distributive justice and fairness of society.

In addition, with increasing use of artificial intelligence, digital technology, and telehealth, there are some potential dangers in the use of background information, personal records and

data, and confidentiality. Recipient institutions and organizations need to be protected against the consequences of these disruptive technologies.

Education and Training

In recent years, medical students, residents, medical schools, and institutions have been showing more enthusiasm for study abroad programs, as well as opportunities to gain experience in a developing country or carry out collaborative research. One study, for example, showed that a significant percentage of the respondents indicated that they had participated in a global mental health experience. A good number of residency programs in the USA have established mental health programs with institutions in Africa and South America. These experiences are useful in providing the participants with a wider view of their field. Consider the comparative and contrastive features of resource allocation, absence of medications, confidentiality practices, treatment approaches, admission criteria, and the cultural variation in the role of family. In a hospital I visited in India, for example, a requirement for admission is that a family member be available to stay in the hospital with the patient. I always recall a conversation in which a young man indicated that he carried out cardiac surgery while on an exchange program. He did not specify what he actually did. If that statement was even true, that practice is indicative of individuals practicing outside of their certification or skills which is an ethical issue in global health and global mental health. Among these are exposure of the student or trainee from an advanced country to low or absent resources, maintaining the visiting trainees' skills to appropriate level of expectations in his/her role in the recipient setting and standards of care. In the planning and execution of exchange programs, it is important to clearly define and articulate the reciprocal nature and expectations in the planning and execution of the project. What are the specific expectations and roles of the trainees, trainer, and faculty members? This is necessary to minimize culture shock and discrepancies of expectations in respect of the

program and the success of the program. Some participants from high-income countries may be dismayed by the level of poverty they may experience in low-income countries. Trainees from non-Western countries may have difficulty with etiquette and cultural differences in perceptions of correct interpersonal space with colleagues. Partnership centers should establish a global mental health curriculum and training agenda that addresses the common issues of age sensitivity and awareness. Sensitive cultural mores around gender and seniority are also issues, for example, when working with Muslim groups. I previously alluded to a middle-aged African chief who found it disagreeable and insulting for a US health worker, younger than his daughter, to attempt to teach him about family planning methods. Many mental health activities and transactions are multidisciplinary. Hence, familiarity with the professional codes of the various disciplines can be helpful.

Ethics and the Humanitarian Space

As previously mentioned, there are currently two traditions of humanitarianism. These are the classical and the resilience-based humanitarian approaches, and as previously indicated, each has their assets and liabilities. In a paper titled *Classical Humanitarianism and Resilience Humanitarianism* making sense of two brands of humanitarian action, Hilhorst cited some major features and perhaps shortcomings of classical humanitarianism. Examples include the strict separation, crisis and normality, and the legal and cultural norms which fence in and restrict classical humanitarianism. Very often in classical humanitarianism, the intervention is seen as a temporary mechanism to deal with the needs generated by the crisis. The interventions tend to be short-lived and are expensive. The actions of the actors are sometimes politicized, and the organizations may become competitive. Although their actions may be motivated by humanitarianism, they may have other agendas. The aid recipients are sometimes seen as victims and potential cheats. This is believed to be at least in part due to the fact that

the recipient population is large and resources are limited. On the other hand, resilience humanitarianism is seen to have continuity between the crisis, development, and peace keeping. The local people and agencies are seen as first responders. Their expertise and knowledge are taken into consideration as assets. Good relations between the agencies and the local people with trust and collaboration are seen to provide more security than adherence to the rigid humanitarian code of classical humanitarianism. In addition, it is perceived that resilience humanitarianism may be less expensive and more effective as it takes into consideration local capacity and resilience. A possible positive feature of resilience humanitarianism is the fact that it may be more flexible. For example, in the area of aids to refugee, the aid can be more appropriately undertaken where the refugees are rather than in a refugee camp. Also, states have been seen to be more assertive and to be playing a bigger role in response to their disasters. Some of these differences between classical and resilience humanitarianism have been ascribed to the fact that it is becoming clear that the major organizations and countries may not be able to respond to the growing prospect of disasters due to climate change.

Ethical Implications for the Future of Global Mental Health

In retrospect, the pre-COVID-19 era was a relatively peaceful and stable set of conditions. Nevertheless, we should not forget our anxieties and fears of continued civil wars and conflicts in urban parts of the world, as well as the consequences of disasters and issues related to food and water insecurity resulting from climate change. Predicting the future even by economists is always hazardous because a single political issue may complicate world stability. Nevertheless, we can venture to discuss this matter without delving into the treacherous and currently difficult to predict consequences of COVID-19.

The political situation even before the advent of COVID-19 continues to make scholars and economists nervous of globalization, and what is

more ominous is the dire and misplaced assertion that global mental health is over. Nevertheless, we can recall the suggestions by Hurst and Thompson that nation states will continue to be the central loci in an increasing complex and globalized world and that they will continue to provide governance to their states (Khan and Najam 2009b). Khan and Najam, in a consultative work for some humanitarian organizations, made some predictions on the future of globalization. They used a scenario-analytic approach. This technique has been cited by the United Nations Environment Programme as the most suitable approach in exploring complex and indeterminate systems such as social or planetary systems (Bono 2012). These systems are characterized by elements of “surprise, ignorance, and violation.” These researchers identified a number of driving forces and applied them to a set of dominant areas. The drivers were (a) information and communication technology, (b) markets, (c) probability, and (d) policy orientation. The domains that were selected were (a) exclusion and inequality, (b) human insecurity, (c) health, (d) cultural and social environment, and (e) institutions and governance. Khan and Najam arrived at three scenarios. These were (a) the Global Marketplace, (b) the Managed Planet, and (c) the Fortress World. The “Global Marketplace” was distinguished by free market and political thinking. The “Managed Planet” was distinguished by dominant policy issues and value systems. The dominant features of the “Fortress World” are a break from current value systems and policy institutions. They predicted that based on the model, the Fortress World will result in increasing marginalization and poverty as well as increased autocracy. It was concluded that the most desirable situation would be a combination of the Global Marketplace and the Managed Planet scenarios. In their report to the international humanitarian group of agencies, Khan and Najam recommended the following strategic approaches. International relief agencies should be strategic in choosing the drivers that best meet their needs and examine how best to achieve their goals. They suggested that these agencies should participate in developing dominant belief systems and

become policy actors and entrepreneurs. These agencies should utilize ICT advances and influence advances in ICT policies. They recommended, in terms of the dominant areas as stated above, that the humanitarian agencies should adopt a strong interest in exclusion and inequity (45). Khan's and Najam's work underscored the need for a framework to explore the issues involved in a complex and over globalized world. These issues include poverty, climate change, migration, and the application of technological advances to the field of global health and global mental health, all very relevant for the international humanitarian agencies especially the faith-based ones. It is perceived that their role will expand with shrinking funds from other sources such as government agencies.

Conclusion

In this chapter we have defined the terms ethics, humanitarianism, and globalization. We gave examples of a number of ethical and humanitarian principles. We addressed some negative consequences of globalization. A definition of global mental health which takes into account a number of criteria and dimensions is suggested. The mesh of these dimensions and criteria with ethical and humanitarian principles can be a minefield for ethical missteps. A number of examples of ethical issues in global mental health are given, and factors likely to be dominant in the future of global mental health are outlined.

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